NCCP 00483 · clinician draft

Nivolumab · every 2 weeks · adjuvant melanoma

Your diagnosis and treatment setting Completely resected melanoma with lymph-node involvement or metastatic disease; the team confirms the stage III or resected stage IV indication. Uveal melanoma is excluded from this adjuvant pathway.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Adjuvant treatment after surgery

Active medicines: Nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic or premedication is specified for this monotherapy. Any symptom-treatment medicine requires an individual authorised prescription.

Additional phase-specific support

- Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.
- Discuss fertility before starting. Use effective contraception during treatment and for at least 5 months after the last dose. Tell the team promptly about a possible pregnancy. Discuss breastfeeding restrictions with the team before treatment; do not breastfeed without their advice. Do not stop prescribed steroids or hormone replacement yourself.
- Immune adverse effects may require corticosteroids and/or long-term hormone replacement under specialist direction. Do not stop prescribed treatment yourself.
- This is an incomplete clinical/pharmacy review draft, not an issued prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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Sources checked 2026-11-06 · SACTCheck v0.84.41 · Regimen-level drafts; prescriber and oncology-pharmacy verification required